When the Youngest Child Ages Out: Caretaker Eligibility and Parental Medicaid Loss

Seeun Park*

November 7, 2026

Abstract

In states that did not expand Medicaid under the Affordable Care Act, low-income parents often qualify through caretaker status, so coverage can end when the youngest child ages out even if the parent's own income has not changed. Using American Community Survey data from 2014 through 2019, I estimate event-study and difference-in-differences models and find that parental Medicaid coverage falls by about eleven percentage points as the youngest child ages out in non-expansion states but remains flat in expansion states, where eligibility is not tied to the presence of a dependent child. Most of the lost coverage becomes uninsurance rather than a shift to another source, and the decline is concentrated among families below their state's parent eligibility line, where parents are most likely to rely on the caretaker pathway. The loss also extends to the aging-out child: the child's own Medicaid coverage falls by more than twice the parental decline and likewise shifts toward uninsurance. A single birthday can therefore increase uninsurance for more than one member of a family.

1 Introduction

In non-expansion states, Medicaid eligibility for many low-income parents is tied to having a dependent child. When the youngest child ages out, that basis for eligibility disappears, and the parent can lose Medicaid even though their own income, health, and circumstances have not changed. A single birthday can end a parent's coverage. This is not an unusual case but a feature of the eligibility structure in these states, exposing a large population of low-income parents to coverage loss. This paper asks what happens to these parents when

*Bowdoin College, Department of Economics. Email: seeunpark03@gmail.com.

the youngest child ages out: whether they lose coverage and, if so, whether they become uninsured or obtain coverage elsewhere.

I answer this question using difference-in-differences (DiD) and event-study designs in the age of the youngest child, applied to pooled American Community Survey data from 2014 through 2019. The design compares parental Medicaid coverage across the youngest child's age, differencing states that did not expand Medicaid (non-expansion states) against states that expanded Medicaid (expansion states) to remove any coverage change generic to a child aging out.

Four results follow. First, parental Medicaid coverage declines as the youngest child ages out in non-expansion states, with no comparable decline in expansion states, providing plausibly exogenous variation in parental coverage. Second, the central result is that most of the lost coverage becomes uninsurance rather than a shift to another source. Among parents below the poverty level, about 89 percent of the Medicaid loss appears as increased uninsurance, consistent with the absence of subsidized Marketplace coverage below 100 percent of the poverty level. Third, the loss is concentrated among parents below their state's parent eligibility line, where caretaker eligibility is available. The pattern therefore follows the eligibility structure, which requires both a dependent child and income below the parent line. Fourth, the loss is a family-level event rather than an individual one. In two-parent households, both parents can lose coverage through the same dependent child, while the aging-out child's own Medicaid coverage falls by more than twice the parental decline and likewise shifts toward uninsurance. A single birthday can therefore increase uninsurance for more than one member of the family.

A large literature documents substantial coverage gains from the Affordable Care Act (ACA) Medicaid expansion, particularly among low-income adults (Courtemanche et al., 2017; Frean, Gruber and Sommers, 2017; Kaestner et al., 2017). Rather than studying the expansion itself, I use the contrast between expansion and non-expansion states as a source of identifying variation. In non-expansion states, eligibility for some parents remains tied to their status as caretakers of a dependent child, whereas in expansion states, low-income parents qualify based on their own income regardless of whether they have a dependent child.

A separate literature exploits age-based eligibility thresholds to study the consequences of gaining or losing public and private benefits, including Medicare eligibility at age 65 (Card, Dobkin and Maestas, 2008, 2009), aging out of parental health insurance coverage (Anderson, Dobkin and Gross, 2012; Dahlen, 2015), and Supplemental Security Income (SSI) reassessment at age 18 (Deshpande, 2016). These studies focus on the individual who crosses the relevant age threshold. This paper instead studies a household spillover in which the child crosses the threshold but the parent loses eligibility. This distinction is particularly

relevant for prime-age adults, who generally face no age-based Medicaid eligibility threshold of their own but may nevertheless experience one indirectly through the age of a dependent child.

The dependence of a parent's Medicaid eligibility on a child is not unique to the aging-out setting. Work on postpartum Medicaid has documented substantial coverage churn around the end of pregnancy-related eligibility (Daw et al., 2017). The mechanism studied here is different. Postpartum coverage is temporary and tied to a pregnancy-related eligibility category, whereas caretaker eligibility continues while a dependent child is present and does not depend on the parent's health. The caretaker basis for eligibility disappears when the youngest child ages out. More broadly, prior work has shown that public insurance coverage can be linked across family members. In Sommers (2006), the linkage operates through enrollment behavior: a child is more likely to remain enrolled in Medicaid or CHIP when a parent is also covered. The linkage studied here is rule-based instead. A child's aging out directly severs the parent's eligibility pathway, so the coverage loss does not depend on the family's enrollment choices.

This setting has two features that make it valuable to study. First, the loss of coverage provides a clean natural experiment. A parent cannot manipulate a child's age in the way they might adjust income to remain below an eligibility threshold or hours to satisfy a work requirement. The aging out of the youngest child is therefore exogenous to the parent's behavior, allowing the resulting coverage loss to be separated from the choices that make many other coverage transitions endogenous. In this respect, the setting resembles the age-based transition into Medicare at 65 (Card, Dobkin and Maestas, 2008, 2009), where eligibility changes with the passage of time rather than with the individual's choices.

Second, the setting reveals how the eligibility structure of non-expansion states operates while it remains in place. In the Medicaid expansion literature, these states typically serve as the comparison group against which the effects of expansion are measured and are therefore characterized largely by what they lack. This paper instead studies that coverage system directly, examining an eligibility structure in which a low-income parent's access to Medicaid can depend on the age of a child rather than solely on the parent's own income or health. For low-income parents subject to this eligibility structure, it is not simply a counterfactual to expansion but the coverage system under which they live.

2 Caretaker eligibility and the aging-out cliff

Medicaid eligibility for non-disabled adults falls into two regimes after the ACA. In expansion states, adults qualify based on income alone up to 138 percent of the federal poverty level. In

non-expansion states, non-disabled adults without a dependent child are generally ineligible for Medicaid, while parents may qualify as Section 1931 caretaker relatives of a dependent child, subject to a state income limit that is typically far below the poverty level.

These parent eligibility limits are low and vary widely across non-expansion states. Table 1 reports the limits as a percentage of the poverty level for a family of three. In 2019, they ranged from 17 percent in Texas and 18 percent in Alabama to 67 percent in South Carolina and 95 percent in Tennessee, with a median near 40 percent (KFF, 2019). Wisconsin is a special case. It covers parents up to 100 percent of the poverty level through a waiver that does not condition eligibility on caretaker status. The aging-out mechanism therefore does not bind there, and I exclude Wisconsin from the treated group.

For caretaker eligibility, a dependent child is generally under age eighteen, or under nineteen if still in school. In a non-expansion state, when the youngest child reaches the relevant age threshold, the parent no longer has a dependent child and loses the basis for caretaker eligibility. This loss is categorical rather than driven by a change in the parent's finances: once the dependent-child condition is no longer met, lower income cannot restore eligibility through the caretaker pathway. If the parent's income is also below 100 percent of the federal poverty level, marketplace subsidies are generally unavailable, leaving the parent in the coverage gap after the youngest child ages out. By contrast, in an expansion state, the same parent can remain eligible for Medicaid based on their own income.

3 Data and sample construction

The data are pooled one-year American Community Survey samples from 2014 through 2019, drawn from IPUMS-USA (Ruggles et al., 2025). I use this window for two reasons. First, it begins after implementation of the ACA Medicaid expansion, so expansion status is well defined. Second, it ends before the pandemic continuous-enrollment requirement, which prevented Medicaid disenrollment from March 2020 through March 2023 and therefore suspended the aging-out mechanism. The one-year ACS files also contain state identifiers, which the design requires.

Seventeen states did not expand Medicaid at any point during the sample period. I exclude Wisconsin because its waiver extended parental eligibility without conditioning on the presence of a dependent child. The treated group therefore consists of sixteen states: Alabama, Florida, Georgia, Idaho, Kansas, Mississippi, Missouri, Nebraska, North Carolina, Oklahoma, South Carolina, South Dakota, Tennessee, Texas, Utah, and Wyoming. The control group consists of the twenty-four states and the District of Columbia that implemented expansion on January 1, 2014 and did not subsequently reverse it, so the caretaker

link is switched off throughout the sample period. States that expanded partway through the window are excluded from both groups to avoid changes in treatment status within the sample.

The unit of analysis is the parent. Because the ACS does not directly identify parents or report the age of a parent's youngest child, I construct these measures using household relationship pointers. Each individual's record includes the person number of their co-resident mother and father, when present in the household (MOMLOC and POPLOC). I invert these pointers to link each parent to their co-resident children. For each parent, I record the age of the youngest linked child, which indexes the aging-out event, along with the number of linked children.

When both parents of the same youngest child are present, I retain the mother because maternal links are more consistently identified. In the sample, 91 percent of children have an identifiable mother in the household, compared with 76 percent who have an identifiable father.[1] I examine the two-parent case directly in Section 5.5, where I measure coverage for both parents rather than retaining only one. A household can nevertheless contain more than one distinct parent-child unit. For example, a young mother may live with her own parents and her child, so both she and her own mother can appear as parents linked to different children within the same household. I address these multigenerational cases through the primary-family restriction described below.

One feature of the ACS income measure shapes the sample construction. ACS income is defined for the Census family and expressed as a percentage of the poverty level. For subfamilies, this measure can incorporate income from the host household even though the parent's Medicaid eligibility unit under Modified Adjusted Gross Income rules consists of the parent and their children rather than the entire household. This mismatch is relatively minor at broad income thresholds but becomes more consequential at tighter cutoffs. I therefore report results for the full linked sample and separately for primary-family parents, defined as the householder or the householder's spouse rather than a member of a subfamily. For these parents, the ACS poverty measure more closely corresponds to their own family unit, so I treat the primary-family sample as the preferred specification.

Coverage is measured using the ACS health insurance variables. The main outcome is an indicator for the parent's own Medicaid coverage. For the coverage decomposition, I also construct indicators for being uninsured and for holding coverage other than Medicaid, so that each parent falls into exactly one of the three categories.

Table 2 reports means for the primary-family analysis sample, consisting of 17,605 non-expansion and 22,730 expansion parents whose youngest child is age fourteen to twenty-two

[1]The main results are similar when the father is retained instead of the mother (Table A1).

and whose family income is below 100 percent of the poverty level. Within this low-income sample, the two groups look broadly similar on household and parent characteristics. The youngest child is about age seventeen in both groups, families have about 1.6 children, the parent is about age forty-seven, and mean income is about 56 percent of the poverty level in each group. Roughly nine in ten parents are women, reflecting the sampling rule that retains the mother when both parents are present. Coverage is where the groups differ most. The share of parents covered by Medicaid is 0.358 in non-expansion states and 0.643 in expansion states, while the uninsured share is correspondingly higher in non-expansion states, 0.342 compared with 0.142. The groups also differ in racial and ethnic composition, with a larger Black share in non-expansion states and a larger Hispanic share in expansion states. These characteristics are reported descriptively rather than used as controls in the main specification because identification rests on differences in age profiles rather than balance in levels.

Figure 1 plots the share of low-income parents with Medicaid by the age of the youngest child in the two groups. In expansion states, the profile is nearly flat across the full age range at about 0.64. In non-expansion states, coverage is lower throughout and declines as the youngest child approaches and passes the aging-out threshold, from about 0.40 among parents whose youngest child is well below the threshold to about 0.27 among those whose youngest child has aged out. The decline is gradual rather than concentrated at a single age because of administrative timing and survey measurement, which I discuss in Section 5. I therefore do not use a regression discontinuity design, which would impose a sharp break that does not match the observed transition and would be sensitive to the precise age at which the cutoff is defined. The empirical analysis instead formalizes the contrast in these age profiles.

4 Empirical strategy

I use a difference-in-differences design across the age of the youngest child, comparing non-expansion and expansion states, to identify the effect of aging out on parental Medicaid coverage. I implement this design using two specifications: an event study and a summary difference-in-differences specification. The event study traces the difference in parental Medicaid coverage between the two groups at each age of the youngest child, allowing the data to reveal the shape of the coverage decline rather than imposing it:

$$Y_{ist} = \sum_{k \neq 17} \beta_k \left(D^k_{ist} \times T_s \right) + \sum_{k \neq 17} \delta_k D^k_{ist} + \alpha_s + \tau_t + \varepsilon_{ist}, \tag{1}$$

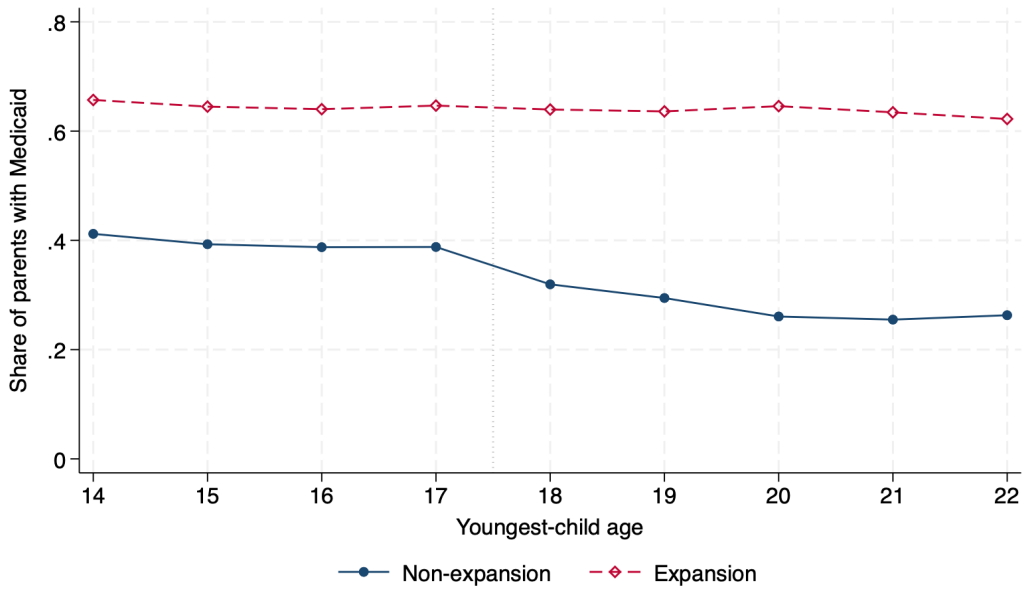


Figure 1: Parental Medicaid coverage by age of the youngest child

Notes: Share of parents covered by Medicaid by age of the youngest child, shown separately for non-expansion and expansion states. Wisconsin is excluded from the non-expansion group. The sample consists of primary-family parents below 100 percent of the federal poverty level whose youngest child is age 14–22, using ACS data from 2014–2019.

where Y_{ist} is an indicator for parent i in state s and year t having Medicaid, T_s indicates a non-expansion state, D_{ist}^k indicates that the youngest child is age k, α_s are state fixed effects, and τ_t are year fixed effects. The state fixed effects absorb the main effect of T_s. Age seventeen is the omitted reference category and is the last age at which the caretaker link remains intact for all families. The δ_k capture the age profile common to both groups, absorbing changes in coverage associated with the youngest child's age regardless of expansion status. The coefficients of interest, β_k, measure the non-expansion–expansion difference at each age relative to the corresponding difference at age seventeen. Because the specification includes a separate indicator for each age, it imposes no functional-form assumption on the age profile and allows the transition around aging out to be observed directly.

The identifying assumption is that, absent the caretaker mechanism, parental Medicaid coverage would evolve with the age of the youngest child similarly in non-expansion and expansion states. The pre-cutoff coefficients provide a direct diagnostic for this assumption. If the two groups were already diverging before the youngest child aged out, the β_k for ages below seventeen would exhibit a systematic pre-trend. Section 5 reports this test.

I also summarize the average coverage change with a difference-in-differences specification:

$$Y_{ist} = \theta \left(A_{ist} \times T_s \right) + \gamma A_{ist} + \alpha_s + \tau_t + \varepsilon_{ist}, \tag{2}$$

where A_{ist} equals zero when the youngest child is age fourteen through seventeen and one when the youngest child is age nineteen through twenty-two. I exclude age eighteen as a donut because the caretaker cutoff can extend to age nineteen for a child who remains in school. As above, the main effect of T_s is absorbed by the state fixed effects. The coefficient θ is the difference-in-differences estimate, comparing the pre- to post-aging-out change in parental Medicaid coverage in non-expansion states with the corresponding change in expansion states. Because the observed coverage loss unfolds gradually across ages, the magnitude of this summary estimate depends on the age window used to define the pre- and post-aging-out periods. I therefore report the estimate across several windows.

The two specifications are complementary: the DiD summarizes the average coverage loss across the pre- and post-aging-out periods, while the event study shows how that loss emerges age by age and provides the pre-trend test.

I estimate both specifications using parents whose youngest child is age fourteen through twenty-two, weight observations by the ACS person weight, and cluster standard errors at the state level. Age eighteen is retained in the event study but excluded from the DiD as described above. Because conventional cluster-robust inference can be unreliable with a small number of state clusters, I also report wild-cluster bootstrap p-values when inference rests on an individual coefficient rather than on the broader pattern of estimates.

5 Results

5.1 Parental Medicaid declines as the youngest child ages out

Figure 2 plots the event-study coefficients β_k from equation (1), which measure the difference in parental Medicaid coverage between non-expansion and expansion states at each age of the youngest child relative to age seventeen. The figure therefore shows both the pre-cutoff pattern underlying the identifying assumption and the coverage loss that emerges after aging out. The sample consists of parents below 100 percent of the poverty level.[2]

The pre-cutoff coefficients provide a test of parallel trends. At ages fourteen, fifteen, and sixteen, the estimated differences are small and statistically insignificant, at 0.009, 0.004, and 0.003. A joint test cannot reject the null that the three coefficients equal zero ($F = 0.10$, $p = 0.96$). Before the caretaker link can be severed, parental Medicaid coverage therefore

[2]The tighter cut below 50 percent used elsewhere for robustness is too sparse in the tails to support the nonparametric age profile. Appendix B shows how the profile changes as the income cutoff is widened.

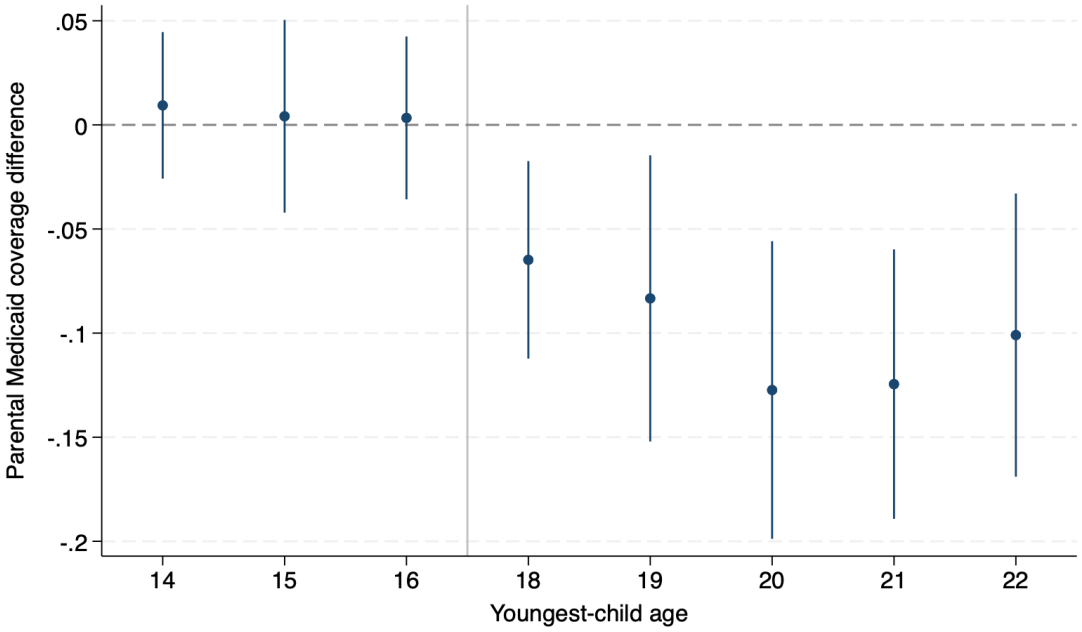


Figure 2: Event-study estimates of parental Medicaid coverage by age of the youngest child

Notes: Each point reports the difference in parental Medicaid coverage between non-expansion and expansion states at a given age of the youngest child relative to age seventeen, estimated from equation (1). Wisconsin is excluded from the non-expansion group. The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019. Bars show 95 percent confidence intervals based on standard errors clustered at the state level.

evolves similarly with the age of the youngest child in the two groups. The flat expansion-state profile in Figure 1 provides the corresponding evidence in levels.

After age seventeen, the difference turns negative and widens. It is -0.065 at age eighteen, -0.083 at age nineteen, and approximately -0.13 at ages twenty and twenty-one, before narrowing somewhat to -0.101 at age twenty-two. The narrowing at the oldest ages may reflect increasing selectivity in co-residence, since a youngest child who still lives with a parent past age twenty-one represents an increasingly selected group. The overall pattern is therefore not a pre-existing divergence but a coverage loss that emerges around the ages at which the caretaker link ends.

Panel A of Table 3 reports the DiD estimates from equation (2). In the full linked sample, the estimate is -0.087 for families below 100 percent of the poverty level and -0.135 for those below 50 percent. Among primary-family parents, the corresponding estimates are -0.112 and -0.176. Because the ACS poverty measure for primary-family parents more closely reflects the parent's own eligibility unit rather than the income of a host household, I use this as the preferred sample in the analysis that follows. The difference between the

two samples is driven by subfamily parents included in the full linked sample. Estimated separately, these parents show little change in coverage at aging out and are substantially more likely to be uninsured even beforehand. Including them therefore attenuates the pooled estimate rather than indicating a weaker effect among parents whose eligibility is tied to the caretaker pathway.

Coverage does not fall to zero after aging out because some parents retain Medicaid through an eligibility pathway that does not depend on a dependent child. The clearest example is Supplemental Security Income, which provides access to a disability-related Medicaid pathway. Table 4 shows essentially no aging-out loss among parents receiving SSI, indicating that the coverage that survives is concentrated among parents with an independent program-based pathway. Functional disability alone does not provide the same protection. Splitting the sample by whether the parent reports any disability in the ACS, I estimate a loss of -0.109 among parents who do and -0.121 among those who do not, which are nearly identical. Most parents reporting a functional difficulty are not enrolled in a disability program and may therefore still depend on the caretaker pathway. The residual Medicaid coverage after aging out thus appears to reflect formal program-based eligibility rather than disability status broadly.

The gradual decline reflects the timing between the child's birthday and the parent's Medicaid redetermination. The ACS records age only in completed years, so age measures time since the relevant cutoff imperfectly. A parent observed when the youngest child is age eighteen, for example, may have crossed the threshold only recently or nearly a year earlier, and coverage ends only after a redetermination occurs. Some parents observed at age eighteen will therefore have lost coverage while others remain enrolled. For children who remain in school, the relevant cutoff occurs at age nineteen, shifting this transition one year later. As time since the cutoff increases, more parents pass through redetermination and the coverage loss becomes more complete. By ages twenty to twenty-one, most affected parents have had time to reach a renewal, and coverage approaches its post-aging-out floor. The observed decline is therefore gradual even though the underlying eligibility change is discrete.

A comparison with the youngest child's own coverage supports this interpretation. Children's Medicaid and CHIP eligibility ends at a single age, nineteen, regardless of school enrollment, so the child's own coverage should fall sharply at that age rather than gradually across several ages. Figure 3 plots the event study for parental Medicaid alongside the same specification estimated for the youngest child's own Medicaid coverage. The child's coverage remains stable through age eighteen and then drops sharply at nineteen, from -0.054 to -0.217, whereas parental coverage declines gradually across ages eighteen through twenty.

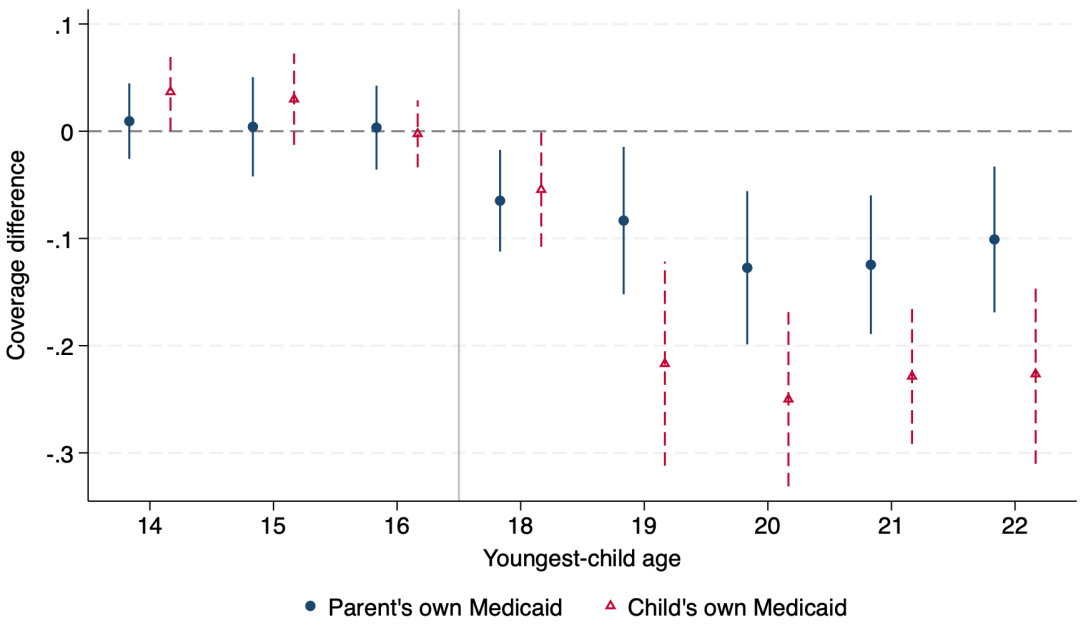


Figure 3: Event-study estimates of parental and youngest-child Medicaid coverage

Notes: Each point reports the difference in coverage between non-expansion and expansion states at a given age of the youngest child, relative to age seventeen, estimated from equation (1). Estimates are shown separately for the parent's own Medicaid coverage (solid circles) and the youngest child's own Medicaid coverage (hollow triangles with dashed confidence intervals). The sample consists of primary-family parents below 100 percent of the federal poverty level, with Wisconsin excluded, using ACS data from 2014–2019. Standard errors are clustered at the state level.

Because the two series come from the same families and the same survey, they are subject to the same whole-year age measurement and interview timing that could blur an age-based transition. Yet the child's transition remains sharp. The more gradual parental decline therefore reflects primarily the caretaker rule itself, which can sever the eligibility link at age eighteen or nineteen depending on the child's school enrollment, rather than survey measurement alone. The parental decline also begins about a year before the child's own coverage loss, as the caretaker mechanism predicts. This timing is inconsistent with an explanation in which parents simply lose coverage alongside the child's own aging out.

5.2 The lost coverage becomes uninsurance

If the decline reflects a genuine loss of eligibility rather than a shift between coverage types, it should appear as increased uninsurance below the poverty level, where affected parents have no subsidized alternative. Figure 4 shows this in two ways. Panel (a) plots the raw coverage shares for non-expansion parents, and panel (b) plots the event-study coefficients for each coverage outcome. The levels in panel (a) show that uninsurance is already high

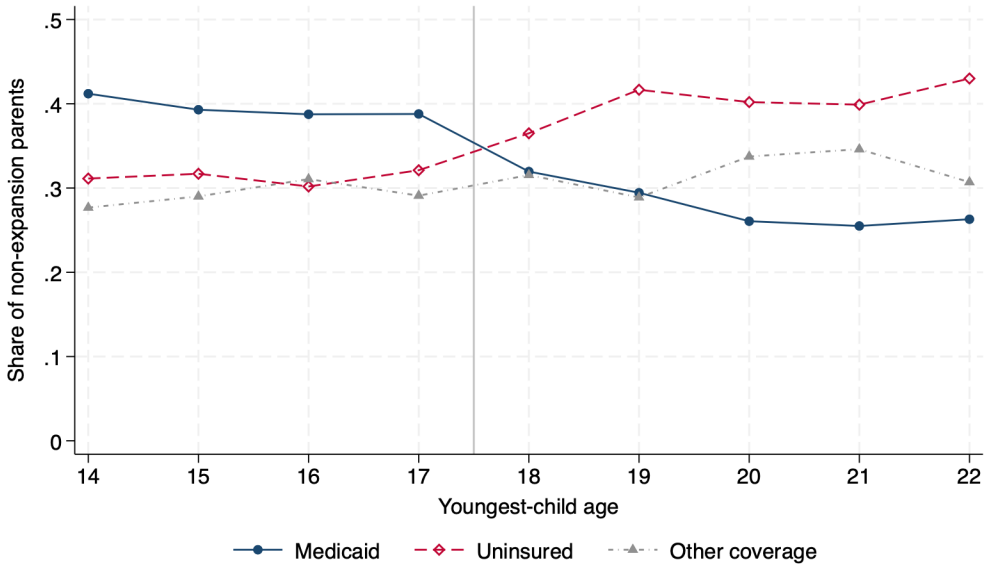
before the youngest child ages out, at about 0.30, so aging out affects a population that is substantially uninsured even beforehand. The event study in panel (b) shows that the decline in Medicaid is mirrored by an increase in uninsurance, while other coverage changes little. The pre-cutoff coefficients are flat for both outcomes. There is therefore little evidence of substitution into other coverage; most of the lost Medicaid appears as uninsurance.

Panel B of Table 3 reports the DiD estimates for each coverage outcome in the primary-family sample. Among parents below 50 percent of the poverty level, the Medicaid loss of -0.176 is matched by a $+0.151$ increase in uninsurance, while other coverage changes by only $+0.025$. The same pattern appears below 100 percent of the poverty level, where Medicaid falls by -0.112, uninsurance rises by $+0.099$, and other coverage changes by $+0.013$. At both income cuts, nearly all of the decline in Medicaid appears as increased uninsurance, with little movement into other coverage. This pattern is consistent with the loss of an eligibility pathway in the coverage gap rather than a reshuffling across sources of insurance. As a share of the Medicaid loss, about 89 percent appears as increased uninsurance below 100 percent of the poverty level and about 86 percent below 50 percent. Because this share divides the estimated increase in uninsurance by the estimated Medicaid loss, sampling error in both estimates produces a wide confidence interval. I therefore treat the point estimate as indicative rather than exact.

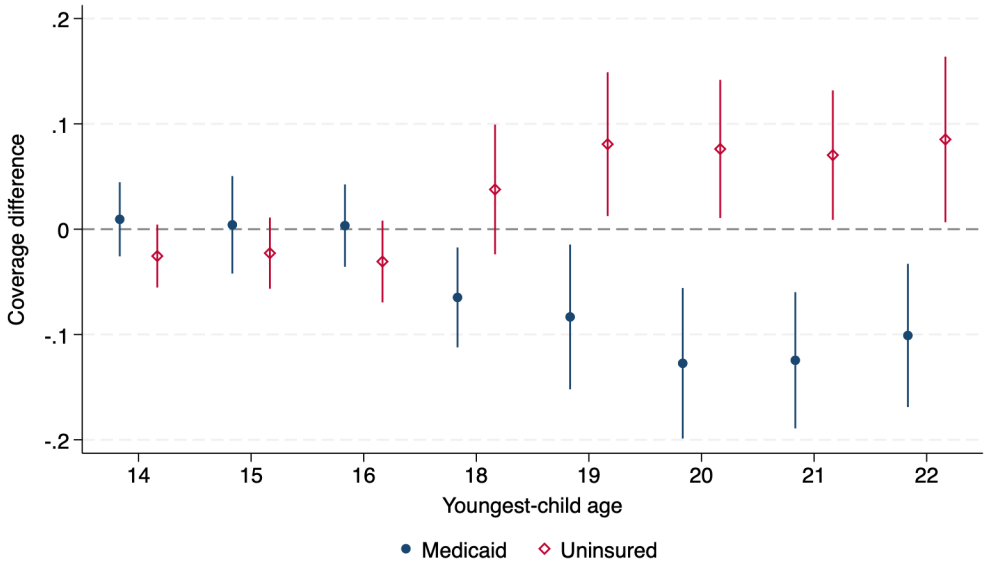
5.3 The loss reaches the aging-out child

The aging-out event affects the child's own coverage as well as the parent's. Children's Medicaid and CHIP eligibility ends at age nineteen regardless of school enrollment, creating a separate coverage transition for the aging-out young adult. Figure 3 shows that the child's own coverage drops sharply at that age rather than gradually. Estimating the difference-in-differences in equation (2) for the youngest child's own insurance, I find that Medicaid coverage falls by -0.247, more than twice the parental decline of -0.112 (Table 5). Most of this loss appears as increased uninsurance rather than other coverage. The increase in uninsurance, 0.209, equals about 85 percent of the Medicaid decline, with only small increases in employer-sponsored and direct-purchase coverage. This share is more precisely estimated for the child than for the parent, with a confidence interval of 77 to 92 percent.

The small increase in private coverage likely reflects the young adult's own entry into the labor market rather than retention on a parent's plan, since these low-income parents rarely hold employer coverage to extend. The contrast with young adults aging out of private parental coverage is instructive. In the pre-ACA setting studied by Anderson, Dobkin and Gross (2012), private dependent coverage often ended around age nineteen, providing a close comparison with the Medicaid and CHIP cutoff studied here. They find that aging out of



(a) Coverage shares (levels), non-expansion parents



(b) Event study, treated minus control

Figure 4: Decomposition of parental coverage by age of the youngest child

Notes: Panel (a) reports raw coverage shares for Medicaid, uninsurance, and other coverage among parents in non-expansion states. Panel (b) reports event-study estimates of the difference between non-expansion and expansion states at each age of the youngest child relative to age seventeen, for Medicaid and uninsurance. Wisconsin is excluded from the non-expansion group. The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019.

private parental coverage reduces insurance coverage by only five to eight percentage points. Dahlen (2015) studies aging out of the ACA dependent-coverage provision at age twenty-six, when employment and own income provide still greater opportunities to substitute toward other coverage, and finds no increase in uninsurance as young adults move into employer-sponsored or directly purchased coverage.

In the low-income population studied here, these substitution margins are much more limited. Aging out of public coverage produces a substantially larger loss, about twenty-five percentage points, and most of it becomes uninsurance rather than shifting to another source. The same aging-out event can also remove the parent's caretaker coverage. A single birthday can therefore increase uninsurance for both the parent and the child, making the family-level consequence larger than the parental estimate alone conveys.

5.4 The coverage loss follows the eligibility structure

The exact eligibility thresholds provide a further test because they define groups with different exposure to parent and child eligibility. Parent eligibility limits lie far below children's limits in every non-expansion state, often by a factor of five or more.[3] This creates three income bands with distinct predictions (Figure 5). Below the parent line, both the parent and child are eligible for public coverage, so aging out should reduce coverage for both (Band 1). Between the parent and child lines, only the child is eligible through these pathways, so only the child should lose coverage (Band 2). Above the child line, neither is eligible through these pathways, so neither should lose coverage (Band 3).

I estimate the DiD separately within each band, allowing each band to have its own age profile. Because expansion states do not share the same eligibility structure, control families are assigned to income-comparable bands using the year-specific median of the treated states' parent thresholds. The bands are therefore eligibility-defined for treated families and income-defined for controls.

Table 6 reports the results. For parents, the estimated loss is -0.209 in Band 1, where they are eligible for caretaker coverage, and -0.032 in Band 2, where they are not. The loss is therefore concentrated where parents are exposed to the caretaker eligibility rule, as the mechanism predicts. The contrast between the two bands confirms that the parent's loss follows the parent's own eligibility. Coverage declines more below the parent line than above it, a difference of -0.169 that is significant under the wild-cluster bootstrap ($p < 0.001$). The small loss above the parent line is itself statistically significant, but it is consistent

[3]The children's line is the combined upper income limit for children's Medicaid and CHIP eligibility, rather than the Medicaid-only limit, because children remain publicly covered through CHIP well above the Medicaid threshold. In 2019, these limits ranged from about 190 percent of the poverty level in Idaho to 317 percent in Alabama.

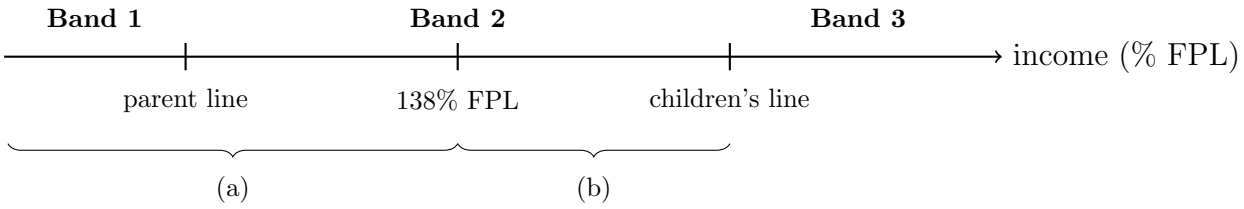


Figure 5: Schematic of income bands defined by parent and child eligibility lines

Notes: Band 1, below the parent eligibility line, is the range in which both the parent and child are eligible for public coverage. Band 2, between the parent and children's eligibility lines, is the range in which only the child is eligible through these pathways. Band 3, above the children's eligibility line, is the range in which neither the parent nor the child is eligible through these pathways. Region (a), below 138 percent of the federal poverty level, is the range in which the child's aging-out effect is identified. Region (b), between 138 percent of the federal poverty level and the children's eligibility line, is the range in which the child's aging-out effect is not identified, because young adults lose coverage in both state groups. Both regions are discussed in the text.

with income misclassification. Because the bands are assigned using annual survey income rather than the monthly income measure used for Medicaid eligibility, some families who were eligible for caretaker coverage may be placed above the parent line and carry a genuine aging-out effect into Band 2. This blurring attenuates the difference between the bands, making the estimated contrast a conservative measure of the underlying eligibility gradient.

The baseline comparison group is Band 2, which runs from the parent line to the children's line. Families in this range carry no parent aging-out effect in either group, although for different reasons. In non-expansion states, parents are already above the caretaker eligibility limit. In expansion states, parental eligibility does not change when the child ages out: parents below 138 percent of the poverty level remain eligible through the adult expansion group, while those above 138 percent are already above its income limit. Restricting the comparison band to families below 138 percent of the poverty level, which roughly halves its size, leaves the contrast essentially unchanged at -0.156 and statistically significant ($p < 0.001$) despite the smaller sample.

The child's coverage loss should not differ between Band 1 and Band 2 because the parent eligibility line does not determine the child's eligibility for public coverage. Children in both bands remain eligible through the child eligibility pathway before aging out. To test this prediction, I restrict Band 2 to families below 138 percent of the poverty level. Below this threshold, an aged-out young adult in an expansion state can transition to adult Medicaid, while the corresponding pathway is absent in non-expansion states. Above 138 percent, young adults in both groups are outside the expansion adult Medicaid income range, so the difference-in-differences would net out any common loss of children's coverage. Within the below-138-percent sample, the child band contrast is small and not statistically significant,

at −0.014. The parent's loss therefore differs sharply across the parent eligibility line, while the child's does not, as implied by the caretaker mechanism.

The band comparison also requires the control group to be well behaved within each band, and it is. In expansion states, parental Medicaid coverage changes little at aging out in any band, with estimated changes of −0.005, −0.013, and +0.006.[4] The flat control profile is therefore present within each band rather than arising from offsetting movements across income groups, so the band contrasts are driven by differences in the treated group rather than movement in the control.

A final test uses an alternative eligibility pathway directly. Parents receiving Supplemental Security Income (SSI) have access to a disability-related Medicaid pathway that does not depend on the presence of a dependent child. Aging out should therefore have little effect on their coverage. Table 4 estimates the aging-out effect separately by SSI receipt. Among parents without SSI, Medicaid coverage declines by -0.129. Among SSI recipients, the estimated change is 0.015 and is not statistically significant, while pre-period coverage is nearly universal at 0.950. The difference between the two groups, 0.155, is statistically significant under the wild-cluster bootstrap. Table A5 shows that this partial decline reflects the composition of pre-period coverage. Coverage associated with disability-related eligibility pathways remains stable when the youngest child ages out, while the decline is concentrated in coverage exposed to the caretaker rule. Together, these results show that the coverage loss is concentrated among parents whose Medicaid eligibility depends on the caretaker pathway, as the mechanism predicts.

5.5 The loss extends to both parents in two-parent households

The main specification measures coverage for one parent per household, the retained mother, and therefore captures only part of the potential household-level loss when the youngest child ages out. In a two-parent family, both parents may qualify through the same dependent child, so a single aging-out event can end coverage for both. To measure this broader effect, I restrict the sample to two-parent families below the poverty level and estimate the same aging-out difference-in-differences using two household-level outcomes: the number of parents covered by Medicaid, ranging from zero to two, and the distribution of households across both parents covered, one parent covered, and neither parent covered.

Table 7 reports the results. Panel A shows that, within two-parent families, an individual parent's Medicaid coverage declines by −0.103, close to the main per-parent estimate, so

[4]These estimates are coefficients on the aging-out indicator from separate regressions of parental Medicaid coverage on the aging-out indicator, state fixed effects, and year fixed effects, estimated within each band among expansion states. Expansion-state observations are assigned to bands using the year-specific median of the treated-state eligibility thresholds, as in the main band analysis.

the result is not an artifact of retaining a single parent. The number of parents covered by Medicaid falls by -0.192, nearly twice the individual-parent effect, indicating that the coverage loss often extends to both parents. Panel B shows that the household distribution shifts in the same direction. The share of families in which both parents are covered falls by -0.121, about 40 percent of its pre-period level of 0.294, with corresponding increases in the one-covered and neither-covered categories. A single aging-out event can therefore remove the eligibility basis for multiple adults in the same household, so the per-parent estimate understates the amount of coverage a family can lose when the youngest child ages out.

5.6 Robustness

Under the caretaker pathway in a non-expansion state, Medicaid eligibility requires two conditions: the presence of a dependent child and family income below the state's parent eligibility line. These are jointly required components of a single eligibility rule, rather than competing explanations for coverage. The relevant concern is therefore not whether income matters for eligibility, but whether changes in income associated with the age of the youngest child could generate a coverage decline that is mistakenly attributed to aging out. I begin with a series of checks addressing this possibility.

Income-related confounding. Figure A1 shows that both family income and the parent's own income evolve similarly with the age of the youngest child in non-expansion and expansion states. Parallel income profiles alone are not sufficient, because the same income change could have different coverage consequences across the two groups. An increase in income could push some non-expansion parents above their relatively low parent eligibility limits while leaving many expansion-state parents eligible up to 138 percent of the poverty level. If such differential exposure were generating the coverage decline, it should also produce divergence in parental Medicaid before the cutoff. The event-study coefficients at ages fourteen through sixteen are small and jointly not statistically significant (Figure 2). The pre-cutoff evidence therefore provides no indication that an income trend is eroding treated coverage before the youngest child ages out.

A second check uses the distance between family income and the parent eligibility line. A family close to the line could cross it after a modest increase in income or a change in measured family size. I therefore re-estimate the aging-out effect among families progressively farther below the line, where increasingly large changes would be required to cross it. Control families, which face no caretaker eligibility line, are assigned the year-specific median treated-state line, as in the band analysis. Table A2 reports the results.

The estimated loss does not weaken with distance below the line. It is −0.21 among all below-line families and remains −0.21 among families at least ten percentage points below the eligibility line. Among one-child families in this range, where the parent's own income rises around aging out relative to multi-child families (Figure A1), the estimate is −0.20. This subgroup therefore provides a particularly informative test of whether income changes can account for the coverage loss. Only seven percent of below-line families lie within three percentage points of the threshold, while the median family is twenty-two points below it. The loss thus persists among families for whom the observed changes in income and family size are too small to explain a crossing of the eligibility line.

Placebo and manipulation checks. A useful placebo replaces the youngest child with an older child. When an older child reaches the aging-out age while a younger dependent child remains, the family may experience many of the same income and household-composition changes associated with a child reaching adulthood, but the parent's caretaker link remains intact. If these accompanying changes generated the Medicaid loss, coverage should decline in this setting as well. It does not. The estimated changes are −0.011 below 100 percent of the poverty level and −0.015 below 50 percent, and neither is statistically significant. Coverage declines when the youngest child ages out and removes the caretaker basis for eligibility, but not when an older child reaches the same age while a younger dependent child remains.

A separate manipulation concern is that parents might have another child as the youngest approaches the threshold, extending the period of caretaker eligibility. Such behavior would alter the distribution of the youngest child's age near the cutoff. Figure A2 instead shows smooth age distributions in both groups, with no localized deficit of non-expansion families near the aging-out ages. The data therefore provide no evidence of family-structure manipulation around the threshold.

I also examine the post-cutoff age profile in the control group. Once treated coverage has fallen, a subsequent decline in expansion-state coverage could mechanically widen the treated-minus-control difference. Parental Medicaid among control families is instead flat over ages nineteen through twenty-two, with a small and statistically insignificant linear slope of −0.004 ($p = 0.53$).[5] The post-cutoff difference therefore does not appear to be driven by an accumulating decline in the control group.

[5]This slope is the coefficient on a linear term in the age of the youngest child from a regression of parental Medicaid coverage on youngest-child age, state fixed effects, and year fixed effects, estimated among expansion-state families over ages nineteen through twenty-two.

Behavioral responses. A further check asks whether parents respond to the coverage loss by adjusting their labor supply. Table A3 reports difference-in-differences estimates for the retained parent's employment, labor-force participation, usual weekly hours, and whether they worked in the past year. None of these outcomes changes appreciably at aging out, and all estimated effects are statistically insignificant. Together with the income profiles above, these results indicate that the coverage loss is not accompanied by an offsetting change in work or income.

Specification and sample robustness. The main estimate is also robust to alternative specifications and treated-group definitions. Because the age of the youngest child is correlated with the parent's age, the estimated effect could in principle capture changes associated with the parent growing older. Adding parent-age fixed effects leaves the estimate essentially unchanged (Table A1). Adding controls for the parent's race and ethnicity likewise has little effect on the estimated coefficient.

The baseline treated group contains sixteen states. Idaho, Nebraska, and Utah approved Medicaid expansion by ballot initiative in 2018, raising the possibility that anticipation of the forthcoming expansions affected enrollment in the final sample year. Excluding these three states produces a thirteen-state treated group with nearly identical event-study estimates (Figure A3) and a similar DiD estimate (Table A1).

Wisconsin is excluded from the baseline because its BadgerCare waiver extends parental Medicaid eligibility to 100 percent of the poverty level without conditioning eligibility on the presence of a dependent child. The caretaker mechanism therefore operates differently there. Including Wisconsin nevertheless has little effect on the estimated coefficient (Table A1).

Finally, the DiD estimates remain negative and statistically significant across a range of age windows around the cutoff (Panel C of Table A1). The estimate remains near -0.11 across the narrower windows and attenuates to -0.095 in the widest window, which includes the oldest post-cutoff ages where the coverage loss narrows, as discussed in Section 5. This variation reflects how the DiD averages over different portions of the age profile. The event study displays that profile directly, and the main result is not driven by a particular choice of pre- and post-cutoff window.

6 Health needs and access to care at the aging-out margin

The ACS results establish that aging out shifts parental coverage from Medicaid toward uninsurance. To characterize the health needs and access to care of the population exposed to this loss, I turn to the 2012–2013 National Health Interview Survey (NHIS), restricted

to the South (Blewett et al., 2026). In this pre-ACA period, low-income parents in these states faced caretaker-based eligibility rules similar to those that remained in place in non-expansion states after 2014.

Parents enrolled in Medicaid report substantial health needs: 58 percent rate their health as fair or poor, compared with 32 percent of uninsured parents. At the same time, Medicaid enrollees report substantially better access to care. Uninsured parents are about three times as likely to lack a usual source of care (40 versus 13 percent) and to report needed medical care they could not afford (44 versus 15 percent).

I focus on self-rated health rather than diagnosed conditions because diagnosis itself depends on access to care and may therefore understate morbidity among the uninsured. These comparisons are based on small samples, 71 Medicaid-enrolled and 99 uninsured parents, and are descriptive rather than causal. They nevertheless help characterize what is at stake when the coverage loss documented in the main analysis occurs: parents with substantial health needs moving from Medicaid toward uninsurance, which in this population is associated with much greater barriers to accessing and affording care.

7 Conclusion

A parent's Medicaid coverage can depend on something that happens to another family member: a child growing up. This paper documents this largely overlooked source of coverage loss in non-expansion states. Parental Medicaid declines when the youngest child ages out, the loss is concentrated below the parent eligibility line, and the lost coverage largely becomes uninsurance. Together, these patterns show that the coverage loss follows the rules governing caretaker eligibility rather than a change in the parent's own income or health.

The loss documented here is a family-level event rather than an individual one. Most research on Medicaid coverage transitions follows a person gaining or losing their own benefit, but caretaker eligibility links the coverage of multiple family members to the age of one child. In a two-parent family, both parents may qualify through the same dependent child, so a single aging-out event can end coverage for both parents at once, as the household results suggest often occurs. The young adult may also lose their own public coverage upon aging out of children's eligibility, a decline more than twice the parental one (Section 5.3). This pattern is consistent with the eligibility structure: in a non-expansion state, a young adult loses children's Medicaid and CHIP at nineteen without a general adult Medicaid pathway replacing it. A single aging-out event can therefore remove Medicaid coverage from multiple members of the same household.

Applying the parental and young-adult estimates to the roughly 35,000 families whose youngest child reaches the threshold each year implies on the order of 13,000 people losing Medicaid annually. Most of these losses are among young adults rather than parents.[6] Viewing the event at the family level reveals the broader reach of the caretaker eligibility structure.

The parents exposed to this loss are among the lowest-income adults served by Medicaid. Nearly 600,000 such parents in non-expansion states are enrolled in Medicaid through the caretaker pathway, and each faces the eventual loss of that eligibility basis as the youngest child ages out.[7] Their incomes fall below their state's parent eligibility line, often far below the poverty level. Many have no subsidized alternative once caretaker eligibility ends because Marketplace premium subsidies generally begin at 100 percent of the poverty level. The loss therefore often becomes a spell of uninsurance. This pattern shows how categorical eligibility rules can end coverage even when a family's financial situation has not otherwise changed, with the losses concentrated among those with few alternatives. Survey evidence reinforces what is at stake: Medicaid-enrolled parents at the aging-out margin have substantial health needs, while uninsured parents in the same population face much greater barriers to accessing and affording care (Section 6).

The mechanism studied here is also becoming more salient for policy. A 2026 federal rule implementing new Medicaid work requirements exempts parents and caretakers of dependent children age thirteen and under. When the youngest child ages out of that exemption, a parent can become subject to new eligibility conditions even if their own employment, income, and health have not changed. The populations and policies are not the same: the new requirement applies primarily to the Medicaid expansion population, and losing the parental exemption does not itself end eligibility. The parents the requirement reaches are low-income caretakers similar to those studied here, and in this sample such parents already work at high rates.[8] If the same holds for the expansion-state parents who lose the exemption,

[6]The annual aging-out cohort is a person-weighted ACS count of parents whose youngest co-resident child is eighteen, below 100 percent of the poverty level, in the non-expansion states excluding Wisconsin, averaged across the six survey years. Applying the parental and young-adult difference-in-differences estimates, -0.112 and -0.247, to this cohort and summing yields about 13,000 people per year. This counts one parent and one young adult per family and therefore understates the total in families where both parents are covered.

[7]This is a person-weighted count from the ACS, 2014–2019, of primary-family parents in the non-expansion states, excluding Wisconsin, who are below 100 percent of the federal poverty level, have a co-resident child under eighteen, and report Medicaid coverage, averaged across the six survey years to an annual national figure. The weighted estimate is approximately 592,000. The number whose Medicaid depends solely on the caretaker link is somewhat smaller, since a minority also qualify through an independent basis such as Supplemental Security Income.

[8]Among the exposed parents, non-expansion, primary-family, below 100 percent of poverty, with a youngest child age fourteen to seventeen, 44 percent are employed and 52 percent worked in the past year. Those who work average 32.7 usual hours per week, and 89 percent of them put in at least eighty hours per

the binding consequence of crossing the age threshold would be administrative rather than behavioral. Those parents would face new requirements to verify work or other qualifying activities, while state Medicaid programs must identify exemptions, verify compliance, issue notices, and process responses. The estimates in this paper therefore do not identify the effect of the new policy, but they highlight a broader question raised by age-based eligibility rules: whether a change triggered by a child's birthday alters behavior or instead creates administrative burden, coverage churn, and potential coverage loss among otherwise eligible families.

Two limitations of the ACS bound the analysis. Age is recorded only in completed years and coverage at a single interview date, while disenrollment occurs at redetermination rather than at the child's birthday. The event study therefore captures the aging-out transition over a range of ages rather than at the precise month of eligibility loss. ACS income is also annual and differs from the Modified Adjusted Gross Income used for monthly Medicaid eligibility, which is likely to attenuate the threshold-based contrasts rather than generate them. The next stage of this project will use individual-level administrative Medicaid data to locate coverage loss more precisely around the aging-out event, trace paths out of and back into Medicaid, and characterize health care use among exposed parents before coverage ends. These data will extend the population-level evidence established here by revealing the timing and individual coverage trajectories underlying the survey patterns.

The administrative extension will deepen the analysis of this particular aging-out mechanism, but the broader lesson of the current paper is already clear. Who keeps Medicaid depends not only on income eligibility, but on how eligibility is structured within the family.

month on their own. These figures understate the share who would meet the requirement, which can also be satisfied through qualifying income, education, or community engagement.

References

Anderson, Michael, Carlos Dobkin, and Tal Gross. 2012. "The Effect of Health Insurance Coverage on the Use of Medical Services." *American Economic Journal: Economic Policy*, 4(1): 1–27.

Blewett, Lynn A., Julia A. Rivera Drew, Andrew Fenelon, Miriam L. King, Kari C.W. Williams, Daniel Backman, Etienne Breton, Grace Cooper, Stephanie Richards, and Renae Rodgers. 2026. "IPUMS Health Surveys: National Health Interview Survey: Version 8.2 [dataset]." *https://doi.org/10.18128/D070.V8.2*.

Card, David, Carlos Dobkin, and Nicole Maestas. 2008. "The Impact of Nearly Universal Insurance Coverage on Health Care Utilization: Evidence from Medicare." *American Economic Review*, 98(5): 2242–2258.

Card, David, Carlos Dobkin, and Nicole Maestas. 2009. "Does Medicare Save Lives?" *Quarterly Journal of Economics*, 124(2): 597–636.

Courtemanche, Charles, James Marton, Benjamin Ukert, Aaron Yelowitz, and Daniela Zapata. 2017. "Early Impacts of the Affordable Care Act on Health Insurance Coverage in Medicaid Expansion and Non-Expansion States." *Journal of Policy Analysis and Management*, 36(1): 178–210.

Dahlen, Heather M. 2015. ""Aging Out" of Dependent Coverage and the Effects on US Labor Market and Health Insurance Choices." *American Journal of Public Health*, 105(S5): S640–S650.

Daw, Jamie R., Laura A. Hatfield, Katherine Swartz, and Benjamin D. Sommers. 2017. "Women in the United States Experience High Rates of Coverage "Churn" in Months Before and After Childbirth." *Health Affairs*, 36(4): 598–606.

Deshpande, Manasi. 2016. "Does Welfare Inhibit Success? The Long-Term Effects of Removing Low-Income Youth from the Disability Rolls." *American Economic Review*, 106(11): 3300–3330.

Frean, Molly, Jonathan Gruber, and Benjamin D. Sommers. 2017. "Premium Subsidies, the Mandate, and Medicaid Expansion: Coverage Effects of the Affordable Care Act." *Journal of Health Economics*, 53: 72–86.

Kaestner, Robert, Bowen Garrett, Jiajia Chen, Anuj Gangopadhyaya, and Caitlyn Fleming. 2017. "Effects of ACA Medicaid Expansions on Health Insurance Coverage and Labor Supply." *Journal of Policy Analysis and Management*, 36(3): 608–642.

Kaiser Family Foundation and Georgetown University Center for Children and Families. 2019. "Medicaid and CHIP Eligibility, Enrollment, and Cost Sharing Policies as of January 2019: Findings from a 50-State Survey." *Kaiser Family Foundation report*.

Ruggles, Steven, Sarah Flood, Matthew Sobek, Daniel Backman, Grace Cooper, Julia A. Rivera Drew, Stephanie Richards, Renae Rodgers, Jonathan Schroeder, and Kari C.W. Williams. 2025. "IPUMS USA: Version 16.0 [dataset]." *https://doi.org/10.18128/D010.V16.0*.

Sommers, Benjamin D. 2006. "Insuring Children or Insuring Families: Do Parental and Sibling Coverage Lead to Improved Retention of Children in Medicaid and CHIP?" *Journal of Health Economics*, 25(6): 1154–1169.

Tables

Table 1: Parent Medicaid eligibility limits in non-expansion states, 2014–2019

State	2014	2015	2016	2017	2018	2019
Alabama	16	18	18	18	18	18
Florida	35	34	34	33	33	32
Georgia	39	38	37	37	36	35
Idaho	27	27	26	26	26	25
Kansas	38	38	38	38	38	38
Mississippi	29	28	27	27	27	26
Missouri	24	23	22	22	22	21
Nebraska	55	55	63	63	63	63
North Carolina	45	45	44	44	43	42
Oklahoma	48	46	44	44	43	42
South Carolina	67	67	67	67	67	67
South Dakota	54	53	52	51	50	49
Tennessee	111	103	101	99	98	95
Texas	19	19	18	18	18	17
Utah	47	46	45	45	60	60
Wisconsin	100	100	100	100	100	100
Wyoming	59	58	57	56	55	54

Notes: Section 1931 parent and caretaker income eligibility limits are reported as a percentage of the federal poverty level for a family of three and measured in January of each year. Data are from the KFF State Health Facts parent eligibility series, based on the KFF and Georgetown CCF 50-state survey. Beginning in 2014, the limits include a disregard equal to five percentage points of the federal poverty level. Wisconsin covers parents up to 100 percent of the federal poverty level through a waiver that does not condition eligibility on caretaker status and is excluded from the treated group.

Table 2: Sample means

	Non-expansion	Expansion
Panel A. Coverage		
Parental Medicaid	0.358	0.643
Uninsured	0.342	0.142
Other coverage	0.300	0.215
Panel B. Household and parent characteristics		
Youngest-child age	16.846	17.006
Number of children	1.585	1.601
Parent age	46.523	47.509
Female	0.909	0.908
Income (% FPL)	56.348	55.998
Panel C. Race and ethnicity		
White	0.377	0.376
Black	0.310	0.186
Hispanic	0.259	0.319
Other race	0.054	0.118
Observations	17,605	22,730

Notes: Means for the primary-family analysis sample, parents with youngest child aged 14–22 and family income below 100 percent of the poverty line, 2014–2019. Non-expansion and expansion states differ in levels, as expected given that non-expansion states are lower-income; the design relies on parallel trends rather than level balance. Race and ethnicity are shown for description; the identification does not require them as controls.

Table 3: Aging-out effects on parental health insurance coverage

	Below 50% FPL		Below 100% FPL	
	Estimate	Pre-mean	Estimate	Pre-mean
Panel A. Parental Medicaid, by sample definition				
Full linked sample	-0.135	0.409	-0.087	0.388
	(0.032)		(0.024)	
	[15,073]		[40,937]	
Primary-family (preferred)	-0.176	0.426	-0.112	0.396
	(0.033)		(0.023)	
	[13,297]		[35,798]	
Panel B. Coverage decomposition, primary-family sample				
Medicaid	-0.176	0.426	-0.112	0.396
	(0.033)		(0.023)	
Uninsured	0.151	0.324	0.099	0.313
	(0.025)		(0.021)	
Other coverage	0.025	0.251	0.013	0.291
	(0.020)		(0.013)	
Observations	13,297		35,798	

Notes: Difference-in-differences estimates compare changes in parental coverage as the youngest child ages out in non-expansion states relative to expansion states, using ACS data from 2014–2019. The sample includes parents whose youngest child is age 14–22. Age 18 is excluded from the DiD as a donut. Estimates are person-weighted, with standard errors clustered at the state level in parentheses and sample sizes in brackets. Panel A reports the Medicaid effect for the full linked sample and the preferred primary-family sample. Primary-family parents are the householder or householder's spouse, for whom the ACS poverty measure more closely reflects the parent's own family unit. Panel B decomposes coverage in the primary-family sample into Medicaid, uninsurance, and other coverage. These three mutually exclusive categories exhaust coverage status, so their estimated effects sum to zero by construction. The Medicaid estimate in Panel B therefore corresponds to the primary-family estimate in Panel A. The pre-period mean is the treated-group mean at youngest-child ages 14–17 for families below the corresponding income cut.

Table 4: Aging-out effect on parental Medicaid by SSI receipt

	Pre-period mean	DiD estimate	Bootstrap p	Obs.
Parents without SSI	0.342	-0.129	< 0.001	32,179
SSI recipients	0.950	0.015	0.478	3,619
Difference (SSI − non-SSI)		0.155	< 0.001	

Notes: Difference-in-differences estimates of the effect of the youngest child aging out on parental Medicaid coverage, estimated separately for parents who report receiving Supplemental Security Income (SSI) and those who do not. SSI receipt is identified from reported SSI income. The difference row reports the interaction between SSI receipt and the aging-out effect. Bootstrap p-values are based on the wild-cluster bootstrap with Webb weights and 9,999 replications, clustered at the state level. The pre-period mean is the treated-group mean at youngest-child ages 14–17. The sample consists of primary-family parents below 100 percent of the federal poverty level, with Wisconsin excluded, using ACS data from 2014–2019. SSI recipients account for 10 percent of the sample.

Table 5: Aging-out effects on the youngest child's own coverage

Coverage outcome	DiD estimate	Pre-period mean	N
Medicaid	-0.247 (0.029)	0.689	35,798
Uninsured	0.209 (0.026)	0.132	35,798
Employer-sponsored	0.023 (0.010)	0.152	35,798
Direct-purchase	0.028 (0.008)	0.063	35,798

Notes: Difference-in-differences estimates of the effect of aging out on the youngest child's own coverage, estimated from equation (2) separately for Medicaid, uninsurance, employer-sponsored coverage, and directly purchased coverage. Standard errors clustered at the state level are reported in parentheses. The pre-period mean is the non-expansion-state mean among families whose youngest child is age fourteen through seventeen. The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019, with Wisconsin excluded.

Table 6: Aging-out effects on Medicaid by eligibility band

	Estimate	Bootstrap p
Panel A. Parent's own Medicaid, full income		
Band 1: below the parent line	-0.209	< 0.001
Band 2: above the parent line	-0.032	0.042
Band 1 versus Band 2	-0.169	< 0.001
Panel B. Contrasts below 138 percent of poverty		
Parent, Band 1 versus Band 2	-0.156	< 0.001
Youngest child, Band 1 versus Band 2	-0.014	0.785

Notes: Difference-in-differences estimates compare aging-out effects in non-expansion states relative to expansion states and are estimated separately within each income band, allowing each band to have its own age profile. The sample is restricted to primary-family parents whose youngest child is age 14–22. Wisconsin is excluded. Band 1 consists of families below the state's Section 1931 parent eligibility limit, where parents are eligible for caretaker coverage. Band 2 consists of families above the parent limit and below the children's eligibility limit, where parents are not eligible through the caretaker pathway. For treated families, bands are defined using each state's parent eligibility limit. Control families are assigned to income-comparable bands using the year-specific median of the treated-state limits. Panel A uses the full income range of Bands 1 and 2. Panel B restricts the comparison to families below 138 percent of the federal poverty level. In this range, an aged-out young adult in an expansion state can transition into the adult Medicaid group, allowing the child contrast to be identified. Bootstrap p-values are based on the wild-cluster bootstrap with Webb weights and 9,999 replications, clustered at the state level.

Table 7: Aging-out effects on parental coverage in two-parent families

	Pre-period mean	DiD estimate
Panel A. Parental Medicaid coverage		
Individual parent's Medicaid	—	-0.103
		(0.023)
Number of parents covered (0–2)	0.648	-0.192
		(0.048)
Panel B. Distribution of parental coverage states		
Both parents covered	0.294	-0.121
		(0.030)
One parent covered	0.060	0.049
		(0.024)
Neither parent covered	0.646	0.071
		(0.023)
Observations		22,441

Notes: Difference-in-differences estimates for two-parent families in non-expansion states relative to expansion states, using the primary-family sample below 100 percent of the federal poverty level from 2014–2019. Standard errors clustered at the state level are reported in parentheses. Panel A reports the effect of the youngest child aging out on an individual parent's Medicaid coverage and on the number of parents covered by Medicaid in the household. The pre-period mean for the latter is calculated at youngest-child ages 14–17. Panel B reports the pre-period share of families in each parental coverage state and the corresponding change at aging out. The three coverage states are mutually exclusive and exhaustive, so the estimated changes sum to zero by construction.

Table 8: Self-rated health and access to care among parents exposed to aging out

	Medicaid	Uninsured
Fair or poor health	58%	32%
Care access		
No usual source of care	13%	40%
Delayed care due to cost	16%	38%
Needed care, could not afford	15%	44%
Could not afford prescriptions	19%	43%
Reduced prescriptions to save	24%	43%
Worried about medical bills	62%	89%

Notes: Weighted shares from the 2012–2013 National Health Interview Survey adult sample. The sample consists of parents ages 25–64 living in the South, with family income below 100 percent of the federal poverty level and a youngest co-resident child age 14–22. Estimates are reported separately for Medicaid-enrolled and uninsured parents. Because this period predates implementation of the ACA Medicaid expansion, low-income parents in these states remained subject to caretaker-based eligibility rules similar to those later retained in non-expansion states. Self-rated health and care-access measures are reported rather than diagnosed conditions because diagnosis itself depends on access to care and may understate morbidity among the uninsured. The comparisons are cross-sectional and do not identify the causal effect of aging out. Cell sizes are 71 Medicaid-enrolled parents and 99 uninsured parents.

Online Appendix

A Robustness Checks

Table A1: Robustness checks for the aging-out effect on parental Medicaid

	Below 50% FPL		Below 100% FPL	
	Estimate	Obs.	Estimate	Obs.
Panel A. Sibling-income channel and caretaker link				
One-child families (size ≤ 3)	-0.174	7,280	-0.119	18,147
	(0.032)		(0.017)	
Older child ages out, link remains	-0.015	17,507	-0.011	46,225
	(0.021)		(0.013)	
Panel B. Specification and sample				
Baseline	-0.176	13,297	-0.112	35,798
	(0.033)		(0.023)	
Parent-age fixed effects	-0.172	13,297	-0.111	35,798
	(0.033)		(0.022)	
Race and ethnicity controls	-0.183	13,297	-0.112	35,798
	(0.034)		(0.023)	
Including Wisconsin	-0.172	13,448	-0.112	36,257
	(0.032)		(0.022)	
Father retained instead of mother	-0.129	14,906	-0.071	40,305
	(0.031)		(0.024)	
Thirteen-state treated group	-0.176	13,129	-0.114	35,327
	(0.033)		(0.023)	
Panel C. Youngest-child age window				
15–21	-0.168	9,834	-0.111	26,563
	(0.031)		(0.024)	
14–22 (baseline)	-0.176	13,297	-0.112	35,798
	(0.033)		(0.023)	
13–23	-0.184	16,779	-0.108	45,279
	(0.036)		(0.026)	
12–24	-0.178	20,375	-0.095	54,849
	(0.035)		(0.026)	

Notes: Difference-in-differences estimates of the effect of the youngest child aging out on parental Medicaid coverage in non-expansion states relative to expansion states, using ACS data from 2014–2019. Standard errors clustered at the state level are reported in parentheses. Panel A examines the sibling-income channel and the role of the caretaker link. The first row restricts the sample to one-child families with a family unit of at most three, thereby excluding households with a co-resident subfamily. The second examines cases in which an older child ages out while a younger dependent child remains, leaving the caretaker link intact. Panel B varies the specification and sample. The baseline excludes Wisconsin because its waiver does not condition parental eligibility on the presence of a dependent child. The thirteen-state specification additionally excludes Idaho, Nebraska, and Utah, which approved Medicaid expansion by ballot initiative in 2018. Panel C varies the youngest-child age window used to define the pre- and post-aging-out periods.

Table A2: Aging-out effects by distance below the parent eligibility line

	All below	Minimum distance below line (pp of FPL)		
	the line	> 3	> 5	≥ 10
Panel A. All families				
Aged-out × non-expansion	-0.209	-0.202	-0.206	-0.209
	(0.027)	(0.025)	(0.026)	(0.027)
Observations	9,614	8,876	8,454	7,624
Panel B. One-child families				
Aged-out × non-expansion	-0.205	-0.196	-0.201	-0.203
	(0.034)	(0.030)	(0.032)	(0.034)
Observations	5,885	5,491	5,256	4,798

Notes: Difference-in-differences estimates of the effect of aging out on parental Medicaid coverage among families below the state parent eligibility line, with the sample progressively restricted by distance below that line. Distance is defined as the state-year parent eligibility limit minus family income, measured in percentage points of the federal poverty level. For example, a family at 22 percent of the poverty level is 18 percentage points below a parent eligibility limit of 40 percent. The restricted columns limit the sample to families more than three, more than five, and at least ten percentage points below the eligibility line, respectively, so progressively larger income changes would be required to cross it. The median below-line family is 22 percentage points below the line, and only 7 percent are within three points. Control families, which do not face a caretaker eligibility line, are assigned the year-specific median parent eligibility limit across treated states. The sample consists of primary-family parents below 100 percent of the federal poverty level, with Wisconsin excluded. Standard errors clustered at the state level are reported in parentheses.

Table A3: Aging-out effects on parental labor supply

Outcome	DiD estimate	Pre-period mean	N
Employed	-0.011 (0.015)	0.442	35,798
In the labor force	-0.023 (0.016)	0.535	35,798
Usual weekly hours	-0.566 (0.668)	16.590	35,798
Worked last year	-0.010 (0.021)	0.516	35,798

Notes: Difference-in-differences estimates of the effect of aging out on the retained parent's labor-supply outcomes, estimated from equation (2) separately for employment, labor-force participation, usual weekly hours, and whether the parent worked in the past year. Standard errors clustered at the state level are reported in parentheses. The pre-period mean is the non-expansion-state mean among families whose youngest child is age fourteen through seventeen. The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019, with Wisconsin excluded.

B Choice of income cut for the event study

The event study in Figure 2 is estimated on the sample below 100 percent of the poverty level rather than the tighter cut below 50 percent used elsewhere for robustness. The reason is that the deep tail of the income distribution is too sparse and too imprecisely measured to support a nonparametric age profile, and this shows up as an apparent violation of parallel trends that is an artifact of the tight sample rather than a feature of the design.

Table A4 makes the point directly. It reports the summary difference-in-differences estimate alongside the pre-cutoff event-study coefficients, at youngest-child ages fourteen, fifteen, and sixteen, and the joint test that they are zero, as the income cut is widened from 50 to 138 percent of the poverty level, the widest being the ACA expansion eligibility ceiling. At the tightest cut the pre-cutoff coefficients are large and the joint test rejects. As the cut widens and the sample grows, the coefficients shrink toward zero and the test moves from rejection to a comfortable failure to reject, reaching $p = 0.96$ at the 100-percent cut used in the paper. The pre-trend decays monotonically with the income threshold and disappears well before the 100-percent cut, which is the signature of small-sample fragility in the deep tail rather than a systematic divergence between the groups. The difference-in-differences estimate is negative and significant at every cut, declining gradually from -0.176 below 50 percent of poverty to -0.112 at the 100-percent cut and -0.075 at the 138-percent ceiling, where the attenuation reflects the addition of higher-income families who were not exposed to the caretaker rule. The result itself is therefore present regardless of where the income threshold is drawn and is concentrated among the lowest-income families where the rule binds. Only the precision of the pre-period age profile is fragile.

Table A4: Pre-trends and the DiD estimate across income thresholds

Income threshold	DiD estimate	β_{14}	β_{15}	β_{16}	Joint F	p	N
$<$ 50% FPL	-0.176 (0.033)	0.121	0.082	0.056	8.36	0.000	15,007
$<$ 65% FPL	-0.138 (0.027)	0.075	0.032	0.012	3.06	0.039	21,778
$<$ 80% FPL	-0.120 (0.024)	0.056	0.009	0.015	2.33	0.089	29,246
$<$ 100% FPL	-0.112 (0.023)	0.009	0.004	0.003	0.10	0.961	40,335
$<$ 138% FPL	-0.075 (0.020)	-0.016	-0.021	-0.001	0.97	0.414	65,832

Notes: Estimates for the primary-family sample at successively wider income thresholds. The 138% FPL threshold corresponds to the ACA Medicaid expansion eligibility ceiling. The DiD estimate is the summary difference-in-differences effect on parental Medicaid coverage, comparing youngest-child ages fourteen to seventeen with nineteen to twenty-two (age eighteen excluded as a donut), with standard errors clustered at the state level in parentheses. The pre-cutoff coefficients β_{14}, β_{15}, and β_{16} are event-study coefficients from Equation 1, estimated relative to age seventeen, and the joint F-test tests whether they are jointly zero. All estimates are person-weighted. N denotes the number of observations in the event-study specification.

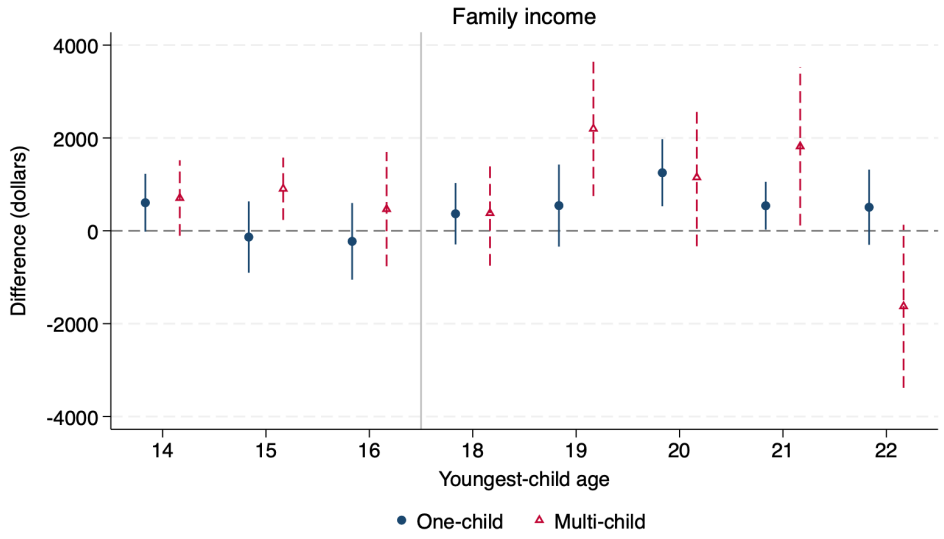
C Composition of the coverage loss

Table A5: Composition of Medicaid coverage before and after aging out

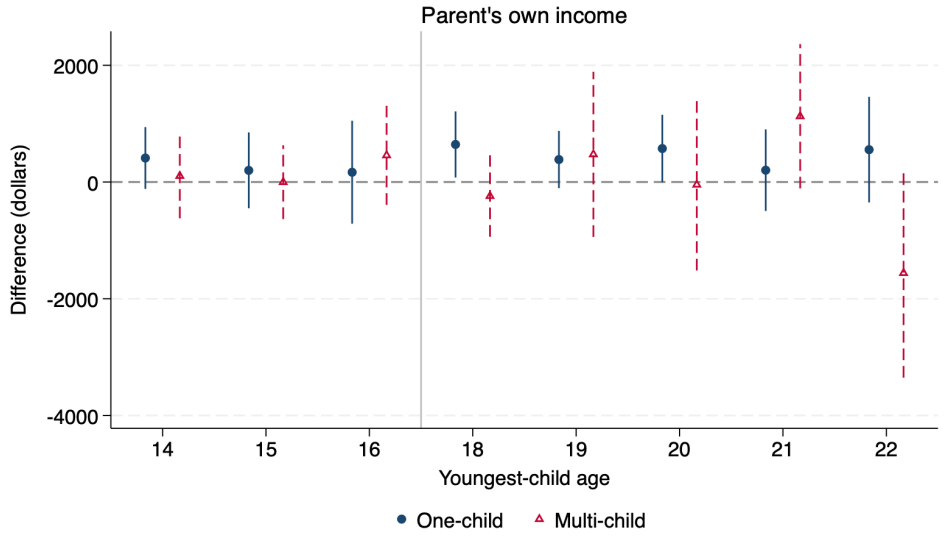
	Disability-linked		Caretaker-only	
Disability-linked coverage defined as	Pre	Post	Pre	Post
SSI receipt	0.084	0.108	0.312	0.163
SSI or Social Security	0.125	0.154	0.271	0.117
SSI, Social Security, or functional disability	0.198	0.194	0.198	0.077

Notes: Weighted Medicaid coverage rates among treated parents in non-expansion states before aging out, when the youngest child is age 14–17, and after aging out, at ages 19–22. Coverage is partitioned into a disability-linked component and a residual caretaker-only component. Each row uses a progressively broader proxy for disability-linked coverage, ranging from SSI receipt alone to SSI, Social Security, or any reported functional difficulty. This is a descriptive decomposition rather than a causal estimate. Total Medicaid coverage in the treated group declines from 0.396 before aging out to 0.271 afterward. Disability-related markers are constructed from reported income sources and ACS functional-difficulty measures. The sample consists of primary-family parents below 100 percent of the federal poverty level, with Wisconsin excluded, using ACS data from 2014–2019.

D Appendix Figures



(a) Family income



(b) Parent's own income

Figure A1: Event-study estimates of income by age of the youngest child

Notes: Panel (a) reports event-study estimates for family income, and panel (b) reports corresponding estimates for the parent's own income. Each point shows the non-expansion–expansion difference at a given age of the youngest child relative to age seventeen. Estimates are shown separately for one-child families (solid circles) and multi-child families (hollow triangles with dashed confidence intervals). The sample consists of primary-family parents below 100 percent of the federal poverty level, with Wisconsin excluded, using ACS data from 2014–2019.

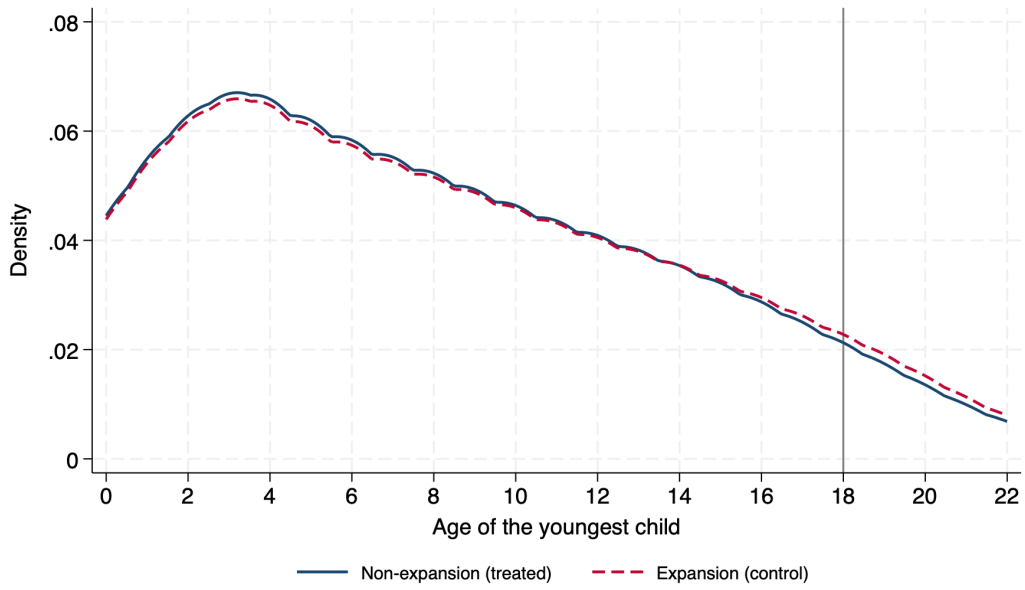


Figure A2: Density of youngest-child age by expansion status

Notes: Kernel density of the age of the youngest child, shown separately for non-expansion and expansion states. The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019, with Wisconsin excluded from the non-expansion group. The vertical line marks the beginning of the aging-out range.

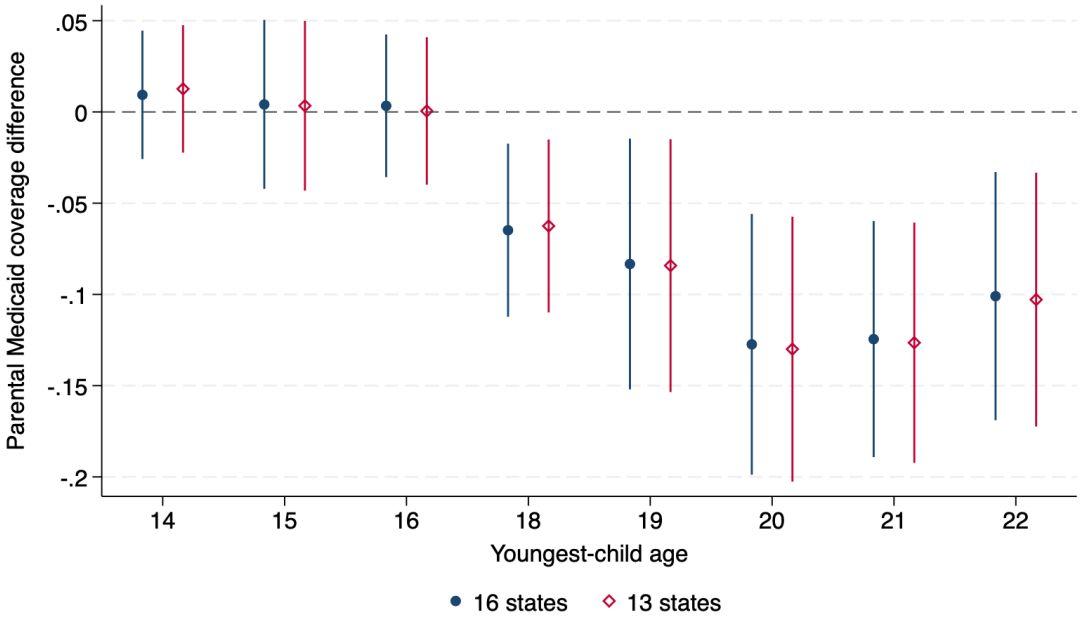


Figure A3: Event-study estimates of parental Medicaid coverage by treated-group definition

Notes: Each point reports the difference in parental Medicaid coverage between non-expansion and expansion states at a given age of the youngest child relative to age seventeen, estimated from equation (1). Estimates are shown for the sixteen-state baseline treated group, which excludes Wisconsin because its waiver extends parental eligibility without conditioning on a dependent child (solid circles), and for the thirteen-state group that additionally excludes Idaho, Nebraska, and Utah, which approved Medicaid expansion by ballot initiative in 2018 (hollow diamonds). The sample consists of primary-family parents below 100 percent of the federal poverty level using ACS data from 2014–2019. Standard errors are clustered at the state level.